REPUBLIC OF THE PHILIPPINES
PHILIPPINE STATISTICS AUTHORITY

REGISTERED DEATHS IN THE PHILIPPINES, 2017

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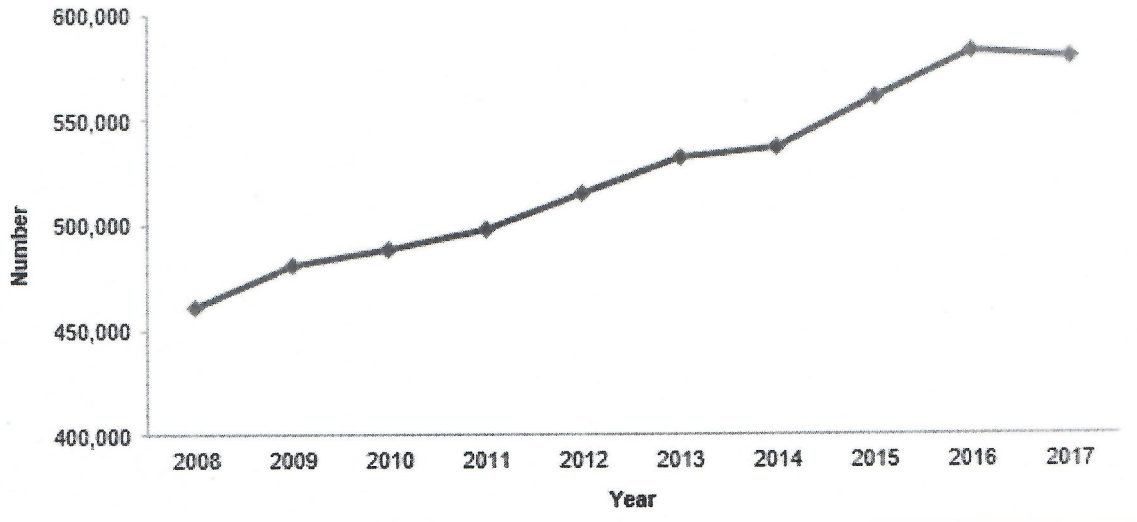
Explanatory Notes

Data on deaths presented in this release were obtained from the Certificates of Death (Municipal Form No. 103) that were registered at the Office of the City/Municipal Civil Registrars all throughout the country and forwarded to the Philippine Statistics Authority. Information presented includes registered deaths which occurred from January to December 2017. Figures presented are not adjusted for under registration.

An average of 1,587 death daily

Reported deaths in 2017 reached 579,237, a decrease of 0.5 percent than the previous year's 582,183 deaths. This is equivalent to a crude death rate (CDR) of 5.5, or about six (6) persons per thousand population. In 2017, an average of 1,587 persons died daily. This translates to 66 deaths per hour or one (1) per minute.

The number of deaths from 2008 to 2016 showed an increasing trend but slightly declined in 2017. The increase during the ten-year period is about a quarter, or 25.5 percent, from 461,581 in 2008 to 579,237 in 2017. (Figure 1 and Table 1).



% Change	4.2	1.5	2.1	3.3	3.4	0.9	4.4	3.8	-0.5

Figure 1. Number and Percent Change of Deaths, Philippines: 2008-2017

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Highest number of deaths in CALABARZON

The top three regions in terms of number of deaths by usual residence were found in Luzon: CALABARZON with 84,971 or 14.7 percent, followed by NCR with 75,187 or 13.0 percent then Central Luzon with 67,980 or 11.7 percent. The combined share of these three regions was 39.4 percent of the total deaths.

On the other hand, the three regions which had the least number of deaths were ARMM (3,036 or 0.5%), CAR (8,176 or 1.4%) and Caraga (14,928 or 2.6%). These numbers accounted for only 4.5 percent of the total deaths in the country (Figure 2 and Table 2).

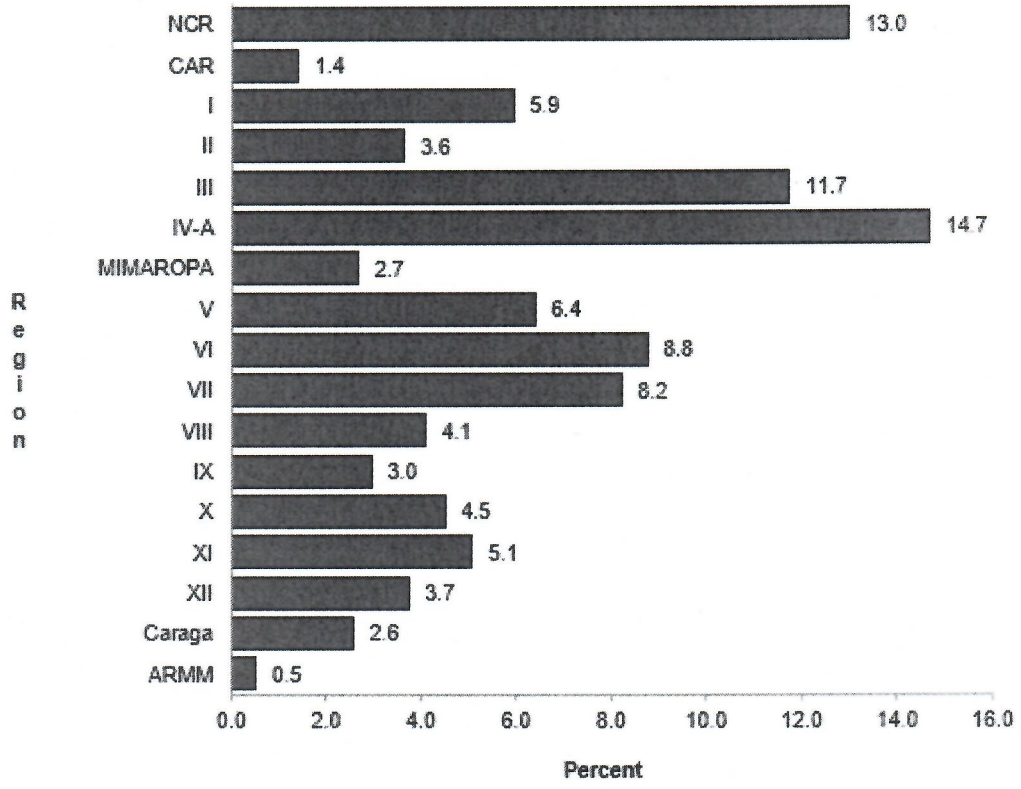


Figure 2. Percent Distribution of Deaths by Region, Philippines: 2017

Most number of deaths in August

The month of August recorded the highest number of deaths with 51,154 or 8.8 percent, while February had the least number with 44,765 or 7.7 percent share of the total deaths.

Daily Index is the increase/decrease from the overall daily average of event occurrences. In 2017, the months of March to July fall below the national daily index of 100.0 (Figure 3 and Table 3).

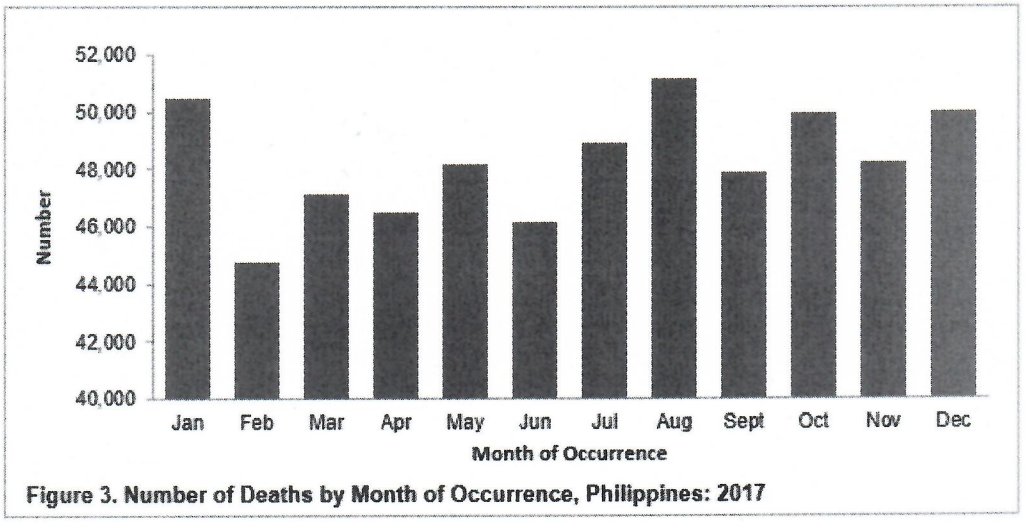


Figure 3. Number of Deaths by Month of Occurrence, Philippines: 2017

More deaths in males than females

In 2017, the number of deaths in males (332,517) was higher than deaths in females (246,720). This translates to a sex ratio of 135, which means that there are 135 male deaths for every 100 female deaths (Table 4).

Figure 4 shows the age and sex pattern of death in 2017. It reflects an inverted pyramid, with fewer deaths at the younger ages, except for children under one, and progressively increasing as people age. As in most parts of the world, males are more likely to die before females at all ages. In the Philippines, it is clearly shown that males died at a higher rate than females before reaching the age of 80 years, with the greatest difference observed at ages 60 to 64 years (15,362 deaths). Higher proportions of female deaths were observed in the older age groups, which is indicative of higher survival rate of females than males.

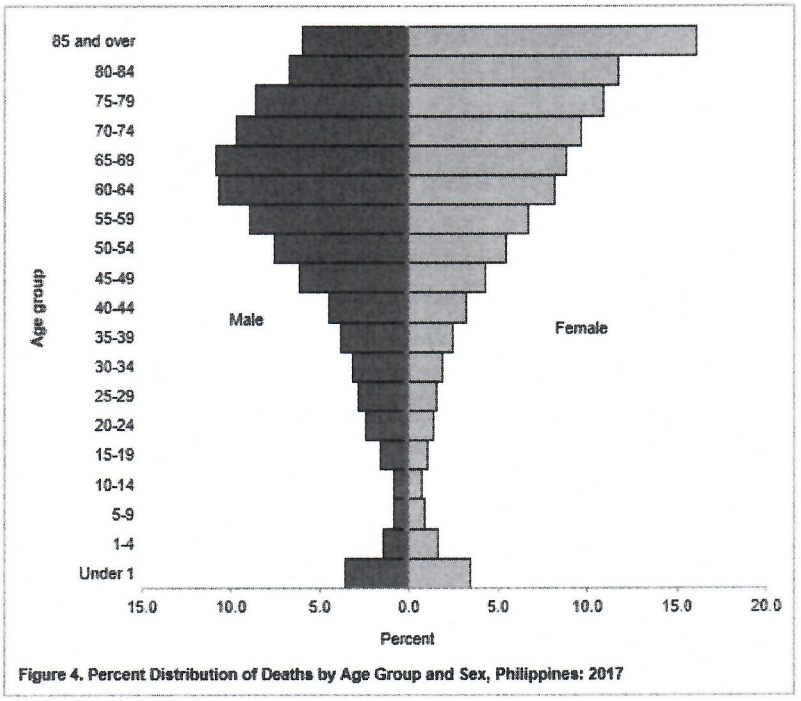


Figure 4. Percent Distribution of Deaths by Age Group and Sex, Philippines: 2017

Six out of ten deaths not attended

Attendant refers to a medical doctor or any other allied health care provider who provided medical attendance to the deceased.

Out of 579,237 registered deaths, about half or 292,098 were not attended. Seven regions namely: NCR (63.4%), CALABARZON (55.4%), CAR (51.7%), Western Visayas (51.5%), Northern Mindanao (51.0%), ARMM (50.7%) and Caraga (50.1%) had more medically attended deaths than those not attended by any health care provider (Figure 5 and Table 5).

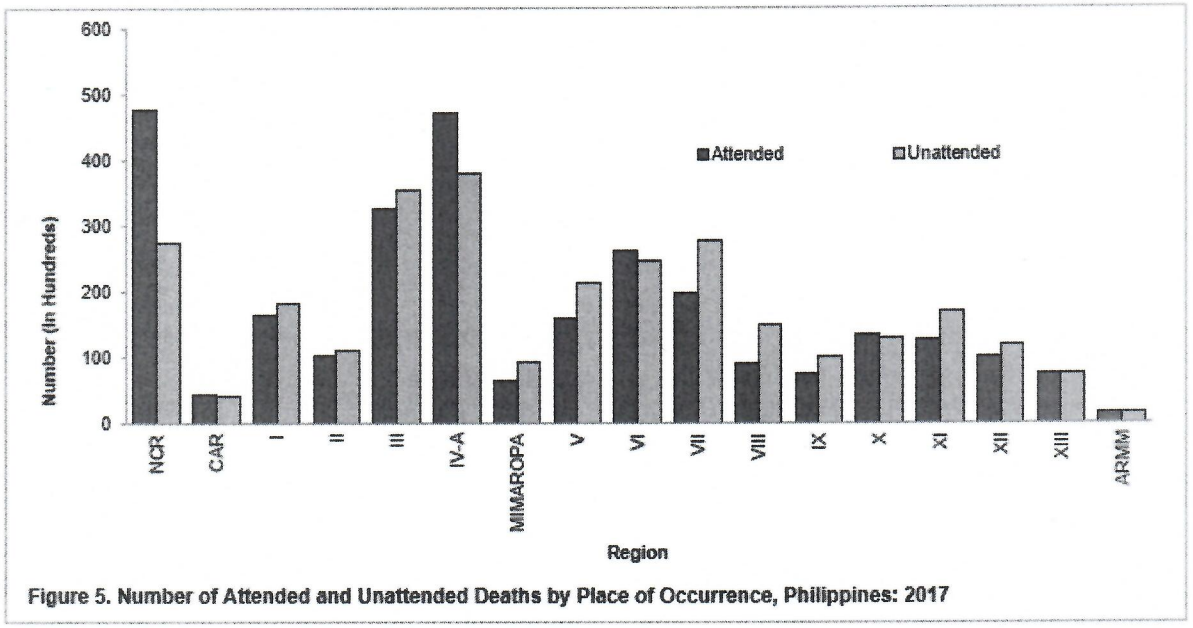


Figure 5. Number of Attended and Unattended Deaths by Place of Occurrence, Philippines: 2017

Highest number of infant deaths in CALABARZON

Infant deaths are deaths that occurred before reaching age 1. At the national level, 20,311 infant deaths were registered in 2017. Six out of ten deaths were males (11,760 or 57.9%). The top three regions that registered high infant deaths were CALABARZON with 3,546 (17.5%), NCR with 3,357 (16.5%) and Central Luzon with 2,314 (11.4%).

On the other hand, ARMM (193 or 1.0%), CAR (205 or 1.0%) and Caraga (386 or 1.9%) had the least number of registered infant deaths (Figure 6 and Table 6).

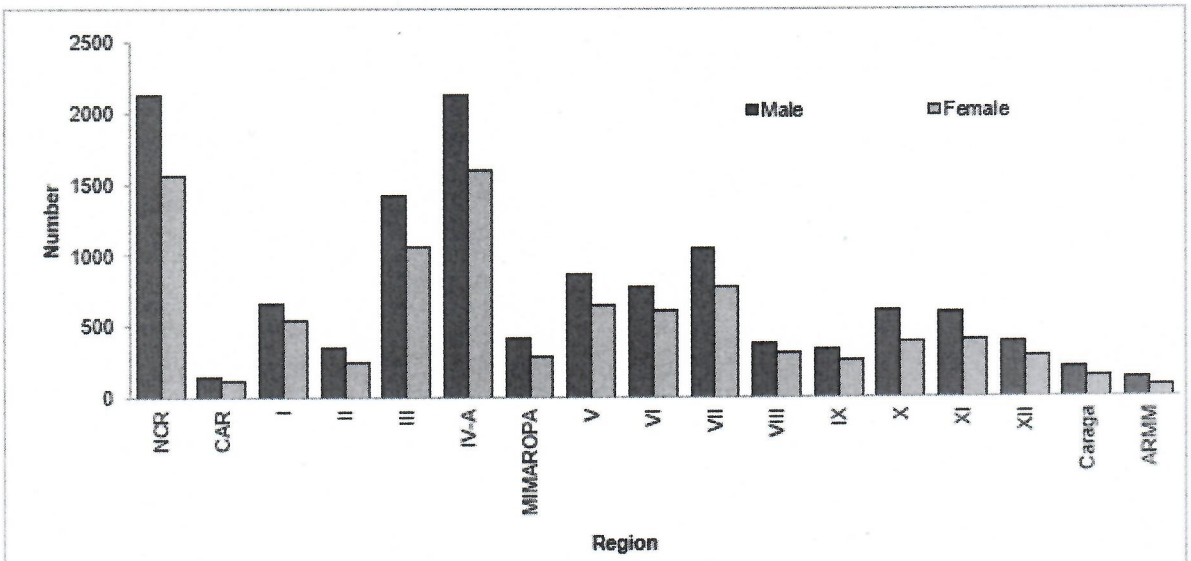


Figure 6. Number of Infant Deaths by Sex and by Region, Philippines: 2017

High maternal deaths in CALABARZON

Maternal death is the death of a woman while pregnant or within 42 days of termination of pregnancy, irrespective of the duration and site of the pregnancy, from any cause related to or aggravated by the pregnancy or its management but not from accidental or incidental causes. In 2017, there were 1,484 registered maternal deaths in the country.

Among all regions, CALABARZON recorded the biggest number of maternal deaths with 222 or 15.0 percent of the total, followed by NCR with 195 or 13.1 percent, and Central Visayas with 164 or 11.1 percent. On the other hand, CAR recorded the least number of maternal deaths with only 13 or 0.9 percent of the total (Figure 7 and Table 7).

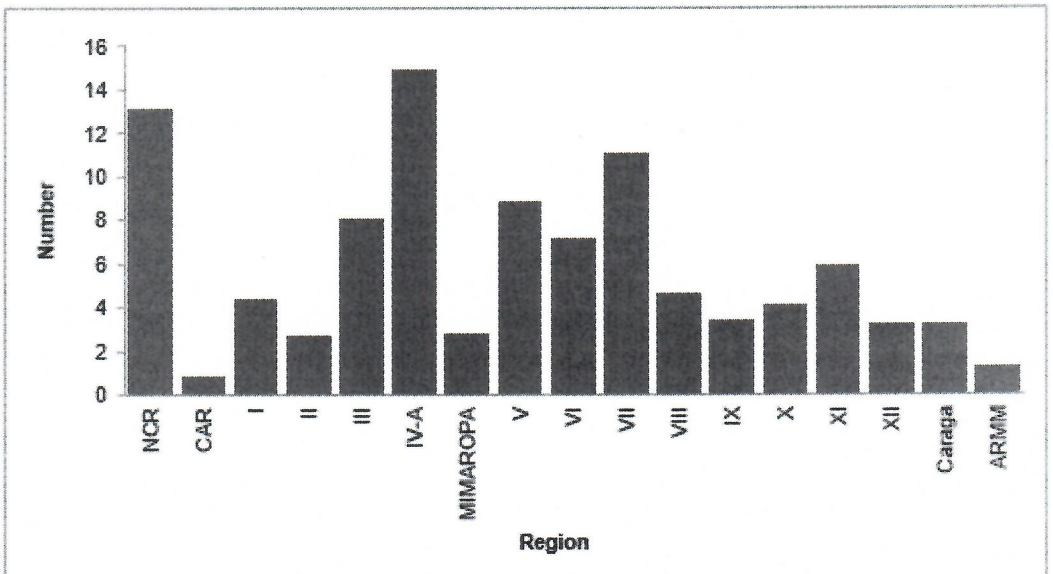


Figure 7. Number of Maternal Deaths by Region, Philippines: 2017

Most deaths due to ischaemic heart diseases

Figure 8 shows the ten leading causes of death in 2017. It can be seen that among the total deaths, ischaemic heart diseases were the leading causes of death with 84,120 or 14.5 percent. Second were neoplasms which are commonly known as "cancer" with 64,125 or 11.1 percent, followed by cerebrovascular diseases with 59,774 or 10.3 percent (Table 8).

Among males, ischaemic heart diseases were also the leading causes of death with 50,503 or 15.2 percent followed by cerebrovascular diseases (33,610 or 10.1%) and neoplasms (30,800 or 9.3%). It was also observed that assault was included in the 10 leading causes of death with 10,866 or 3.3 percent of the total deaths in males. On the other hand, similar to males, the top cause of death among females was also ischaemic heart disease (33,617 or 13.6%), followed by neoplasm with 33,325 or 13.5 percent and pneumonia with 28,835 or 11.7 percent of the total deaths in females (Table 8).

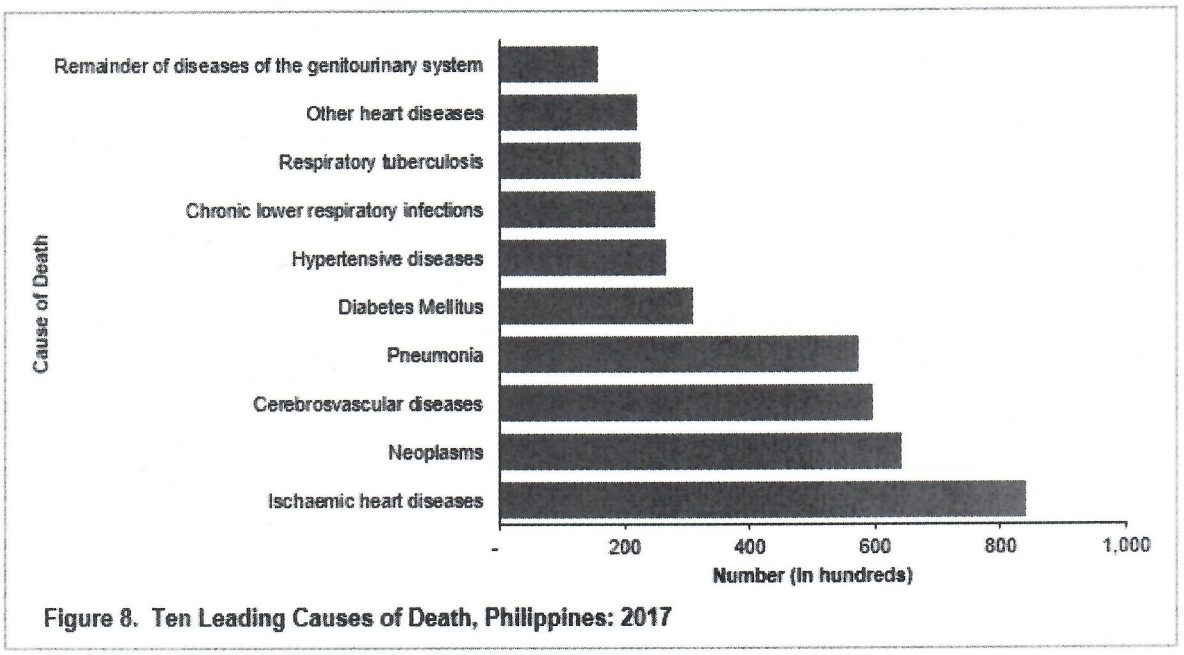


Figure 8. Ten Leading Causes of Death, Philippines: 2017

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Table 1. Number of Deaths and Percent Change: Philippines 2008-2017

	2008	2009	2010	2011	2012	2013	2014	2015	2016	2017
Number	461,581	480,820	488,265	498,486	514,745	532,176	536,999	560,605	582,183	579,237
Per Day	1,265	1,317	1,338	1,366	1,410	1,458	1,471	1,536	1,591	1,587
Per Hour	53	55	56	57	59	61	61	64	66	66
Percent Change		4.2	1.5	2.1	3.3	3.4	0.9	4.4	3.8	(0.5)

Source: Philippine Statistics Authority, Civil Registration Service, Vital Statistics Division
Note: Figures are results of actual registration without any adjustment for under registration

Table 2. Number and Percent Distribution of Deaths by Sex and by Usual Residence (Region), Philippines: 2017

Usual Residence (Region)	Number			Percent			Sex Ratio
	Both Sexes	Male	Female	Both Sexes	Male	Female	
Philippines	579,237	32,517	46,720	100.0	57.4	42.6	134.8
National Capital Region	75,187	43,100	32,087	13.0	7.4	5.5	134.3
Cordillera Administrative Region	8,176	4,748	3,428	1.4	0.8	0.6	138.5
Region I - Ilocos Region	34,427	19,244	15,183	5.9	3.3	2.6	126.7
Region II - Cagayan Valley	21,115	12,398	8,717	3.6	2.1	1.5	142.2
Region III - Central Luzon	67,980	38,656	29,324	11.7	6.7	5.1	131.8
Region IV-A - CALABARZON	84,971	48,735	36,236	14.7	8.4	6.3	134.5
MIMAROPA Region	15,489	9,046	6,443	2.7	1.6	1.1	140.4
Region V - Bicol	37,052	21,071	15,981	6.4	3.6	2.8	131.9
Region VI - Western Visayas	50,701	29,045	21,656	8.8	5.0	3.7	134.1
Region VII - Central Visayas	47,575	26,807	20,768	8.2	4.6	3.6	129.1
Region VIII - Eastern Visayas	23,721	13,331	10,390	4.1	2.3	1.8	128.3
Region IX - Zamboanga Peninsula	17,123	9,882	7,241	3.0	1.7	1.3	136.5
Region X - Northern Mindanao	26,065	15,055	11,010	4.5	2.6	1.9	136.7
Region XI - Davao	29,415	17,478	11,937	5.1	3.0	2.1	146.4
Region XII - SOCCSKSARGEN	21,693	13,112	8,581	3.7	2.3	1.5	152.8
Region XIII - Caraga	14,928	8,590	6,338	2.6	1.5	1.1	135.5
Autonomous Region in Muslim Mindanao	3,036	1,793	1,243	0.5	0.3	0.2	144.2
Foreign Countries	583	426	157	0.1	0.1	*	271.3

Source: Philippine Statistics Authority, Civil Registration Service, Vital Statistics Division
Note: Figures are results of actual registration without any adjustment for under registration
** Less than 0.05 percent*

Table 3. Number and Percent Distribution of Deaths, Daily Average and Daily Index, by Month, Philippines: 2017

Month	Number	Percent	Daily Average	Daily Index
Total	579,237	100.0	1,587	100.0
January	50,476	8.7	1,628	102.6
February	44,765	7.7	1,599	100.7
March	47,136	8.1	1,521	95.8
April	46,487	8.0	1,550	97.6
May	48,141	8.3	1,553	97.9
June	46,118	8.0	1,537	96.9
July	48,913	8.4	1,578	99.4
August	51,154	8.8	1,650	104.0
September	47,870	8.3	1,596	100.5
October	49,922	8.6	1,610	101.5
November	48,225	8.3	1,608	101.3
December	50,030	8.6	1,614	101.7

Source: Philippine Statistics Authority, Civil Registration Service, Vital Statistics Division
Note: Figures are results of actual registration without any adjustment for under registration

Table 4. Number and Percent Distribution of Deaths by Sex and Age Group, Philippines: 2017

Age Group	Both Sexes		Male		Female	
	Number	Percent	Number	Percent	Number	Percent
Total	579,237	100.0	332,517	100.0	246,720	100.0
Under 1	20,311	3.5	11,760	3.5	8,551	3.5
1-4	8,573	1.5	4,612	1.4	3,961	1.6
5-9	5,095	0.9	2,895	0.9	2,200	0.9
10-14	4,594	0.8	2,713	0.8	1,881	0.8
15-19	8,072	1.4	5,362	1.6	2,710	1.1
20-24	11,392	2.0	7,903	2.4	3,489	1.4
25-29	13,174	2.3	9,404	2.8	3,770	1.5
30-34	15,066	2.6	10,449	3.1	4,617	1.9
35-39	18,647	3.2	12,566	3.8	6,081	2.5
40-44	22,693	3.9	4,866	4.5	7,827	3.2
45-49	30,808	5.3	20,277	6.1	10,531	4.3
50-54	38,570	6.7	25,147	7.6	13,423	5.4
55-59	46,151	8.0	29,749	8.9	16,402	6.6
60-64	55,504	9.6	35,433	10.7	20,071	8.1
65-69	57,660	10.0	35,939	10.8	21,721	8.8
70-74	56,172	9.7	32,299	9.7	23,873	9.7
75-79	55,506	9.6	28,651	8.6	26,855	10.9
80-84	51,396	8.9	22,408	6.7	28,988	11.7
85 and over	59,677	10.3	19,954	6.0	39,723	16.1
Not Stated	176	*	130	*	46	*

Source: Philippine Statistics Authority, Civil Registration Service, Vital Statistics Division
Note: Figures are results of actual registration without any adjustment for under registration
** Less than 0.05 percent*

Table 5. Number and Percent Distribution of Deaths by Attendant and Usual Residence (Region) Philippines: 2017

Usual Residence (Region)	Total	Attended		Unattended	
		Number	Percent	Number	Percent
Philippines	579,237	287,139	49.6	292,098	50.4
National Capital Region	75,187	47,694	63.4	27,493	36.6
Cordillera Administrative Region	8,176	4,224	51.7	3,952	48.3
Region I - Ilocos Region	34,427	16,367	47.5	18,060	52.5
Region II - Cagayan Valley	21,115	10,143	48.0	10,972	52.0
Region III - Central Luzon	67,980	32,487	47.8	35,493	52.2
Region IV-A - CALABARZON	84,971	47,090	55.4	37,881	44.6
MIMAROPA Region	15,489	6,390	41.3	9,099	58.7
Region V - Bicol	37,052	15,738	42.5	21,314	57.5
Region VI - Western Visayas	50,701	26,107	51.5	24,594	48.5
Region VII - Central Visayas	47,575	19,787	41.6	27,788	58.4
Region VIII - Eastern Visayas	23,721	8,796	37.1	14,925	62.9
Region IX - Zamboanga Peninsula	17,123	7,256	42.4	9,867	57.6
Region X - Northern Mindanao	26,065	13,288	51.0	12,777	49.0
Region XI - Davao	29,415	12,578	42.8	16,837	57.2
Region XII - SOCCSKSARGEN	21,693	9,868	45.5	11,825	54.5
Region XIII - Caraga	14,928	7,478	50.1	7,450	49.9
Autonomous Region in Muslim Mindanao	3,036	1,538	50.7	1,498	49.3
Foreign Countries	583	310	53.2	273	46.8

Source: Philippine Statistics Authority, Civil Registration Service, Vital Statistics Division
Note: Figures are results of actual registration without any adjustment for under registration

Table 6. Number and Percent Distribution of Infant Deaths by Usual Residence, by Sex, Philippines: 2017

Region	Number			Percent		
	Both Sexes	Male	Female	Both Sexes	Male	Female
Philippines	20,311	11,760	8,551	100.0	57.9	42.1
National Capital Region	3,357	1,933	1,424	16.5	9.5	7.0
Cordillera Administrative Region	205	133	72	1.0	0.7	0.4
Region I - Ilocos Region	1,136	668	468	5.6	3.3	2.3
Region II - Cagayan Valley	590	347	243	2.9	1.7	1.2
Region III - Central Luzon	2,314	1,346	968	11.4	6.6	4.8
Region IV-A - CALABARZON	3,546	2,078	1,468	17.5	10.2	7.2
MIMAROPA Region	653	378	275	3.2	1.9	1.4
Region V - Bicol	1,364	760	604	6.7	3.7	3.0
Region VI - Western Visayas	1,321	773	548	6.5	3.8	2.7
Region VII - Central Visayas	1,632	938	694	8.0	4.6	3.4
Region VIII - Eastern Visayas	547	307	240	2.7	1.5	1.2
Region IX - Zamboanga Peninsula	483	289	194	2.4	1.4	1.0
Region X - Northern Mindanao	959	563	396	4.7	2.8	1.9
Region XI - Davao	988	561	427	4.9	2.8	2.1
Region XII - SOCCSKSARGEN	630	369	261	3.1	1.8	1.3
Region XIII - Caraga	386	206	180	1.9	1.0	0.9
Autonomous Region in Muslim Mindanao	193	108	85	1.0	0.5	0.4
Foreign Countries	7	3	4	*	*	*

Source: Philippine Statistics Authority, Civil Registration Service, Vital Statistics Division
Note: Figures are results of actual registration without any adjustment for under registration
** Less than 0.05 percent*

Table 7. Number and Percent Distribution of Maternal Deaths by Usual Residence (Region), by Sex, Philippines: 2017

Usual Residence (Region)	Number	Percent
Philippines	1,484	100.0
National Capital Region	195	13.1
Cordillera Administrative Region	13	0.9
Region I - Ilocos Region	65	4.4
Region II - Cagayan Valley	41	2.8
Region III - Central Luzon	120	8.1
Region IV-A - CALABARZON	222	15.0
MIMAROPA Region	42	2.8
Region V - Bicol	131	8.8
Region VI - Western Visayas	106	7.1
Region VII - Central Visayas	164	11.1
Region VIII - Eastern Visayas	69	4.6
Region IX - Zamboanga Peninsula	51	3.4
Region X - Northern Mindanao	62	4.2
Region XI - Davao	88	5.9
Region XII - SOCCSKSARGEN	48	3.2
Region XIII - Caraga	48	3.2
Autonomous Region in Muslim Mindanao	19	1.3

Source: Philippine Statistics Authority, Civil Registration Service, Vital Statistics Division
Note: Figures are results of actual registration without any adjustment for under registration

Table 8. Ten Leading Causes of Death by Sex, Philippines: 2017

Leading Cause/Sex	Number	Percent
All causes of death	579,237	100.0
Ischaemic heart diseases	84,120	14.5
Neoplasms	64,125	11.1
Cerebrovascular diseases	59,774	10.3
Pneumonia	57,210	9.9
Diabetes Mellitus	30,932	5.3
Hypertensive diseases	26,471	4.6
Chronic lower respiratory infections	24,818	4.3
Respiratory tuberculosis	22,523	3.9
Other heart diseases	22,134	3.8
Remainder of diseases of the genitourinary system	15,717	2.7
Other causes of death	171,413	29.6
MALE		
All causes of death	332,517	100.0
Ischaemic heart diseases	50,503	15.2
Cerebrovascular diseases	33,610	10.1
Neoplasms	30,800	9.3
Pneumonia	28,375	8.5
Chronic lower respiratory infections	17,633	5.3
Respiratory tuberculosis	15,689	4.7
Diabetes Mellitus	15,164	4.6
Hypertensive diseases	14,061	4.2
Other heart diseases	11,672	3.5
Assault	10,866	3.3
Other causes of death	104,144	31.3
FEMALE		
All causes of death	246,720	100.0
Ischaemic heart diseases	33,617	13.6
Neoplasms	33,325	13.5
Pneumonia	28,835	11.7
Cerebrovascular diseases	26,164	10.6
Diabetes Mellitus	15,768	6.4
Hypertensive diseases	12,410	5.0
Other heart diseases	10,462	4.2
Chronic lower respiratory infections	7,185	2.9
Respiratory tuberculosis	6,834	2.8
Remainder of diseases of the genitourinary system	6,521	2.6
Other causes of death	65,599	26.6

Source: Philippine Statistics Authority, Civil Registration Service, Vital Statistics Division
Note: Figures are results of actual registration without any adjustment for under registration
Causes of death are coded based on the ICD-10 Rules and Guidelines
Symptoms, signs and abnormal clinical and laboratory findings, not elsewhere classified (R00-R99) were not included in the top 10 causes of death

TECHNICAL NOTE

PSA IMPLEMENTATION OF IRIS SOFTWARE: UNDERSTANDING CODING AND PROCESS IMPROVEMENTS

In 2017 the Philippines Statistical Authority (PSA) implemented Iris, an automated software program which assigns codes from the International Classification of Diseases 10th Revision (ICD-10) to death records, and assists in the selection of an underlying cause of death. This replaces the previous process where mortality coding rules were applied manually.

The implementation of Iris has enabled the PSA to update mortality coding processes and mortality statistical outputs, bringing the coding of Philippines mortality data up to date with international best practice.

The Iris implementation project is part of a large scale initiative the Government of Philippines has undertaken with the Bloomberg Philanthropies Data for Health/Vital Strategies initiative to improve civil registration and vital statistics in the Philippines. As part of this project a number of initiatives have been undertaken which relate directly to death certification, registration and data analytics. These initiatives include:

- Courses in medical certification for doctors across major hospitals in the Philippines;
- Campaigns to improve death registration rates;
- Training in medical certification for cause of death coders and data analysts; and
- Training in ICD-10 coding rules as governed by the World Health Organization.

The move to Iris, alongside the additional CRVS initiatives being undertaken in the Philippines all have the potential to impact statistical outputs. It should be noted that updates applied to the ICD-10 are regulated by the World Health Organization (WHO) and are adopted only where they enhance accuracy or reflect improved medical understanding. To that end the changes in this issue will improve the quality of the Philippines' mortality dataset.

The purpose of this Technical Note is to provide a summary of changes that have occurred as a result of software and coding updates. This technical note provides a resource for data users to understand where changes have occurred within the dataset and the impact of those changes moving forward.

PSA INVESTIGATIONS INTO IRIS

PSA investigations into Iris were conducted throughout 2016 and 2017. A key element of these investigations was a dual coding exercise conducted where records were coded through Iris and manually. Approximately 200,000 records from the 2016 and 2017 reference year were processed through both techniques (Iris and manual) with outputs being analysed thoroughly. Results showed that Iris would provide a strong platform for future autocoding and enable best practice in mortality coding and statistical output to be sustained. The Iris software is language independent, and as such is being used extensively around the world. Updates made to ICD-10 by the WHO are rapidly implemented in Iris, meaning that cause of death coding practices and statistical outputs will remain up to date.

KEY CHANGES IN OUTPUT DUE TO IRIS IMPLEMENTATION

The PSA is committed to communicating to data users how Iris has changed statistical output. This technical note will assist users in understanding what may be a legitimate change in disease process as opposed to administrative by-products of recent changes to the death certification, registration and analytical system.

The PSA annually publishes the leading causes of death in the Philippines, with output based on Mortality Tabulation List 1 of the ICD-10 of suggested statistical outputs. The following analysis will be based on this leading cause of death tabulation. For details on other changes to coding and statistical output please contact the PSA-VSD.

LEADING CAUSES OF DEATH, 2017

The leading cause of death in 2017 in the Philippines was ischaemic heart disease, followed by neoplasms and cerebrovascular diseases (see table below). Although the ischaemic heart diseases were the leading cause of death in 2016, the numerical change is statistically significant.

LEADING CAUSES OF DEATH – TIME SERIES WITH NUMBER

Cause of Death	2013	2014	2015	2016	2017
Ischaemic heart diseases	65,378	65,551	68,572	74,134	84,120
Neoplasms	53,601	55,588	58,715	60,470	64,125
Cerebrovascular diseases	54,578	52,894	58,310	56,938	59,774
Pneumonia	53,101	53,689	49,595	57,809	57,210
Diabetes Mellitus	27,064	31,539	34,050	33,295	30,932
Hypertensive diseases	29,067	34,902	34,506	33,452	26,471
Chronic lower respiratory infections	23,867	24,686	23,760	24,365	24,818
Respiratory tuberculosis	22,013	23,157	24,644	24,462	22,523
Other heart diseases	33,027	34,141	31,729	28,641	22,134
Remainder of diseases of the genitourinary system	16,785	17,220	18,061	19,759	15,717

A key coding change with the introduction of Iris has been in regards to ischaemic heart disease and hypertension. Prior practice based on local coding rules was to combine hypertension and ischaemic heart diseases into a hypertensive heart disease regardless of placement of the two diseases on the death certificate. With the implementation of Iris, ischaemic heart disease is retained as the underlying cause of death in many of these cases, meaning the number of deaths coded to I20-I25 (ischaemic heart diseases) has increased and the number of deaths coded to hypertensive diseases (I10-I15) has decreased.

In addition to ischaemic heart diseases, hypertensive diseases have also been reallocated to cerebrovascular diseases with the implementation of Iris. Hypertension is a key risk factor for strokes, especially those of haemorrhagic aetiology, and there has been an increase in these conditions (I60-I62) under the new coding model.

Tuberculosis has decreased as an underlying cause of death under the implementation of Iris. There is a tendency to select tuberculosis as the underlying cause of death regardless of position placement on the death certificate under the former manual coding rules. This has now changed and the tuberculosis must legitimately have initiated the train of morbid events leading to death to be assigned as the underlying cause of death. It is also acknowledged that the Department of Health (DOH) have implemented many prevention and intervention campaigns targeted at tuberculosis in recent years. This may also have

legitimately reduced number of deaths due to tuberculosis. As the administrative and real world changes are difficult to separate, caution should be taken when interpreting time series data for tuberculosis.

LEADING CAUSES OF DEATH, 2017, BY STANDARDIZED DEATH RATE

The prior analysis of change is based on numerical differences over a time series seen in the PSA data. To assess the impact of change at a population level data should be presented as a death rate. A death rate provides a number of deaths as expected per 100,000 density in a population and helps take into account changes in population numbers and age structure of time. The table below shows the top 10 causes of death in the Philippines over the last 5 years as standardized death rates.

LEADING CAUSES OF DEATH – TIME SERIES WITH RATES

Cause of Death	2013	2014	2015	2016	2017
Ischaemic heart diseases	66.6	65.6	67.5	71.8	80.2
Neoplasms	54.6	55.7	57.8	58.6	61.1
Cerebrovascular diseases	55.6	53.0	57.4	55.1	57.0
Pneumonia	54.1	53.8	48.8	56.0	54.5
Diabetes Mellitus	27.6	31.6	33.5	32.2	29.5
Hypertensive diseases	29.6	34.9	34.0	32.4	25.2
Chronic lower respiratory infections	24.3	24.7	23.4	23.6	23.7
Respiratory tuberculosis	22.4	23.2	24.3	23.7	21.5
Other heart diseases	33.6	34.2	31.2	27.7	21.1
Remainder of diseases of the genitourinary system	17.1	17.2	17.8	19.1	15.0

When presented as death rates, we can see that the proportional changes from 2016 to 2017 are not as large as suggested by numerical changes in the first table. It is recommended that policy makers and researchers use rates in conjunction with numbers of deaths to provide context to an area of interest.

OTHER NOTABLE AREAS OF CHANGE WITH THE IRIS IMPLEMENTATION

Although the focus of this technical note has been on the leading causes of death in the Philippines in 2017 there are general areas of change which should be highlighted in order to demonstrate the higher level of quality in statistical output for causes of death with this release. Notable changes are discussed below:

Decrease in specified causes of death considered "direct": There has been a decrease in deaths due to cardiac arrest and respiratory failure as the underlying cause of death in Iris. These conditions are considered ill-defined and do not indicate where health funding and intervention should be targeted. These deaths are now coded to more specific diseases appearing on the death certificate.

Improved identification of primary and secondary cancers: There has been an increase in cancers of unknown primary cause with the implementation of Iris. This is due to an improved ability to identify secondary cancers and code them accordingly. Under previous rules, cancers were generally assumed to be primary in the majority of cases leading to an over-count of particular cancer types.

More consistent distribution of deaths due to external causes: Under prior coding rules there was a lack of internal business rules on how to code the intent (i.e. accident, suicide,

homicide) of deaths due to injury where no intent was stated by the certifier. A set of business rules have been implemented as part of the Iris project and there is greater consistency in the output for external causes of death.

MOVING FORWARD

The PSA remains committed to maximising the relevance and useability of mortality data in the Philippines, ensuring alignment with international best practice for mortality coding and maintaining international comparability. To that end, this project will also instigate changes and improvements in statistical output for causes of death in the Philippines.

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	For appropriate action	Dear Sir For your further comments/ endorsement if ok. Thnx Ann R.
	For comments	
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	For drafting of reply	
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Date: 6/6/2019 Time: To: SIR NS DENNIS M. From: OIC TAGHAP	Action Needed	Remarks/Notes
	For appropriate action	We are endorsing herewith the Special Release on Death cited 2017 for your signature and approval. Thanks. RCT
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		For drafting of reply	
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Date: 06 JUNE Time: To: Ma'am Au From: Nerry	Action Needed		Remarks/Notes
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